Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

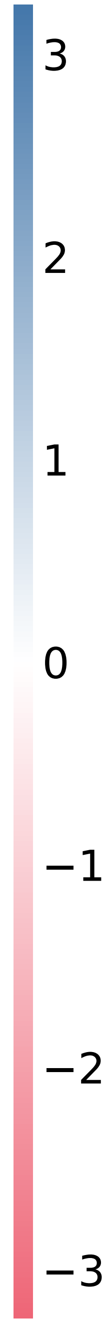
Answer Choices:

A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

C. Pneumonia

D. Congenital heart anomaly with right-to-left shunt



Reasoning + Answer:

<th ink > The symptoms mentioned like t ach yp nea , sub cost al and inter cost al retr actions , nasal fl aring , cyan osis , and responsive to oxygen are indicative of respiratory distress . Additionally , the baby was born after the 3 2 nd gest ational week , implying it is not NR DS (Ne on atal respiratory distress syndrome) due to it being more common in premature babies . Tr ache oes ophage al fist ula is not typically associated with such symptoms . P ne um onia would often be associated with a more localized presentation . Cong en ital heart anomalies with a right -to -left sh unt can cause respiratory distress , especially with the signs of hyp ox ia and cyan osis . </ think >< answer >C ong en ital heart anomaly with right -to -left sh unt </ answer >